

BREAST SYMPTOMS

Approach to the patient with a breast symptom who is not breastfeeding

Breast lump/s			Breast pain	Nipple discharge/retraction	Breast enlargement
Any of: patient > 25 years, family history of breast cancer, irregular fixed lump, skin/nipple changes, nipple discharge or axillary lymph node?			- Reassure that pain is unlikely due to breast cancer. - If lump/s, see adjacent. - Exclude pregnancy ↗ 157.		
Yes	No				
Refer same week to a breast clinic/Regional Breast Unit.	One breast	Both breasts			
	Re-examine breast on day 7 of menstrual cycle. If lump persists, refer to breast clinic/Regional Breast Unit within 21 days.	Fibrocystic change likely - Pain usually occurs before period and improves with period. - Reassure this is common and advise a well-fitting bra. - If pain, give paracetamol 1g 4-6 hourly (up to 4g in 24 hours) as needed with food for up to 5 days. - May be a side effect of hormonal contraception. If no better after 3 months on contraception, change method ↗ 154. - Advise to return if symptoms change/worsen: refer to breast clinic/Regional Breast Unit within 60 days.			
			- Refer to breast clinic/Regional Breast Unit same week if any of: - Blood-stained - One-sided discharge - Patient ≥ 50 years - Male - Skin/nipple changes - Breast/axillary lump - If pregnant, reassure and give antenatal care ↗ 160. - Review medication: antipsychotics, antidepressants, oral contraceptive and metoclopramide can cause nipple discharge. Discuss with doctor. - If cause uncertain, refer.		
			- If only one breast enlarging, refer to breast clinic/Regional Breast Unit. - Check if this is obesity. If BMI¹ > 25 assess CVD risk ↗ 127. - Review medication: antipsychotics, antidepressants, efavirenz, nifedipine, amlodipine can cause breast enlargement. Discuss with doctor. If on efavirenz, doctor to consider switching medication ↗ 117.		

Approach to the patient with a breast symptom who is breastfeeding

Painful/cracked nipples	Painful breast/s <i>without</i> lump		Painful breast/s <i>with</i> lump		
• Usually due to poor latching: help to latch baby properly. • Avoid using soap on nipples. • Advise to apply breastmilk to nipples after feeding and expose to air. Apply zinc and castor oil ointment between feeds.	Temperature ≥ 38°C or body pain?		Temperature ≥ 38°C or body pain?		
	Yes: mastitis likely • Give flucloxacillin³ 500mg 6 hourly for 5 days and paracetamol 1g 4-6 hourly (up to 4g in 24 hours) as needed for up to 5 days. • Advise warm compresses. • If no better after 2 days or breast lump (abscess) develops, refer.		No	No	Yes
			Engorgement likely	Blocked duct likely	Breast abscess likely
• If HIV negative, advise to continue breastfeeding. • If HIV positive: if only one breast affected, express and discard milk from this side. Continue breastfeeding from other side. - If both breasts affected, advise to temporarily stop feeding from breast, express, heat-treat² milk, and cup-feed baby until cracks/mastitis resolve. If heat treating not possible: explore circumstances at home to assess safety of formula feeding. If barriers to safe formula feeding, advise to continue breastfeeding and emphasize importance of strict ART adherence and viral suppression.			• Advise frequent breastfeeds, warm compresses and to gently massage breast. • Advise to return if fever/body pain develops or if breast lump persists: consider other causes and discuss/refer.	Refer same day.	
Refer to breastfeeding counsellor/lactation consultant or support group. If HIV positive, give routine HIV care ↗ 111 and prevent transmission to baby ↗ 168.					

¹BMI = weight (kg) ÷ height (m) ÷ height (m). ²Heat-treat milk to rid it of HIV and bacteria: place breastmilk in sterilized glass jar. Close lid and place in pot. Fill pot with water 2cm above milk and heat water. Remove jar when water is rapidly boiling. ³If severe penicillin allergy (history of anaphylaxis, urticaria or angioedema), give instead **azithromycin** 500mg daily for 3 days.